



GUIDELINE

Sexual Safety

Scope (Staff):	All Staff, students & volunteers
Scope (Area):	CAMHS

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Only use this box if relevant (e.g. clinical document or available on an external website)

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Aim

The aims of these guidelines are:

- To establish and promote a culture that is protective of sexual safety;
- To ensure staff are aware of the practices that will keep young people safe from sexual harm; and
- To ensure staff are aware of the steps they should take to maintain appropriate boundaries and minimise the risk of allegations of abuse.

Risk

The following groups are at particular risk of sexual abuse:

- Children with impaired judgement (including cognitive impairment from mental illness);
- Young females who have a history of physical and sexual abuse;
- Young people of diverse sexuality, sex and/or gender; and
- Children who are heavily sedated.

While children and young people may be vulnerable to sexual safety breaches they may also pose a sexual risk to others.

Background

The CAMHS Sexual Safety Guideline provides a framework to facilitate the development of a culture and service environment within CAHS CAMHS to promote, protect and support the sexual safety of children and families.

These guidelines apply to all CAMHS staff including PCH, Community and Specialised services and are inclusive of clinical, administration, corporate and support staff, peer workers, students, volunteers and managers.

All children, families, and staff members have a right to be safe from harm. The protection of the human rights of people with mental illness is a key objective of the [Mental Health Act 2014](#) and the [Chief Psychiatrist WA Standards of Clinical Care 2015](#). CAMHS is committed to meeting the Charter of Mental Health Care Principles when providing treatment, care and support to children and families.

These guidelines are not intended to guide staff in responding to an allegation of abuse. The documents listed below provide specific instructions in response to incidents of concern:

- [Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service Policy](#)
- [Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist;](#)
- [WA Health Notifiable and Reportable Conduct Policy](#)

- [WA Health Complaint Management Policy \(2015\)](#)
- The [Children and Community Services Act 2004](#), prescribes mandatory reporters (including doctors and nurses) to make a report when they have a reasonable belief that child sexual abuse has occurred.
- The [CAHS Child Safety and Protection Policy](#) provides governance and direction around child protection issues.
- The [Mandatory Reporting of Child Sexual Abuse Training Policy](#) establishes the minimum requirements for CAHS to support staff to meet their legislative reporting requirements as they relate to child sexual abuse.

CAMHS would like to acknowledge the Office of the Chief Psychiatrist (OCP) and the [Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia](#) which have been utilised as a tool in the development of this guideline.

Definitions

Sexual Safety: refers to being and feeling psychologically and physically safe, including being free of, and feeling safe from, behaviour of a sexual nature that is unwanted, or makes another person feel uncomfortable, afraid or unsafe. This includes sexual assault and harassment. It also extends to being spoken to using sexualised language or observing other people behaving in a sexually disinhibited manner, including nakedness and exposure or masturbation, being made to watch or shown pornographic images and lacking privacy and dignity when naked.¹

Incidents that compromise sexual safety include the types of behaviour that can breach and/or compromise the sexual safety of a child and include:

- Sexual assault or harassment (by fellow patients, visitors or staff);
- Consensual sexual activity in an inappropriate context or setting (such as inpatient or day service); and
- Sexually disinhibited behaviour.

Childhood sexual abuse: occurs when an adult, adolescent or child uses their power or authority to involve a child in sexual activity. It includes sexual behaviour where:

- (a) A child is bribed, coerced, threatened, exploited or forced; or
- (b) A child has less power than the person/s involved; or
- (c) There is a significant difference in the child's developmental age or maturity and that of the other person involved.

Sexual assault: a broad term used to describe a range of sexual acts committed against a person without their consent. It occurs when a person is forced, coerced or tricked into sexual acts against their will or without their consent.² According to the [WA Criminal Code \(1913\)](#):

- A child under the age of thirteen years is incapable of giving consent to any sexual activity or exposure.
 - Sexual activity with a child aged under sixteen years, but over thirteen years of age is illegal, unless the age difference between the two people involved is less than three years, and the other party can show that they had reasonable grounds to believe that the child was over sixteen years of age.
 - Sexual activity between a child under eighteen years but over sixteen years of age and any adult who provides care, supervision or authority of the child is illegal (e.g. health practitioner, step-parent, guardian, foster parent, employer, teacher, coach, priest, etc).

Sexual assault is a crime of violence with serious consequences. Conviction for sexual assault is punishable by law and depending on the category of assault, can carry a jail term. [The Criminal Code 1913](#) sets out offences of a sexual nature in Part 5, Chapter 31.

Sexual harassment: defined as ‘unwelcome conduct of a sexual nature which makes a person feel offended, humiliated and/or intimidated where that reaction is reasonable in the circumstances’. Sexual harassment can involve conduct such as:

- Unwelcome touching, hugging or kissing, staring or leering; or
- Suggestive or offensive comments or jokes; or
- Unwanted invitations to go out on dates or requests for sex; or
- Intrusive questions about an individual’s private life or body; or
- Unnecessary familiarity, such as deliberately brushing up against someone; or
- Insults or taunts of a sexual nature; or
- Sexually explicit emails or SMS messages; or
- Behaviour which would also be an offence under the criminal law, such as physical assault, indecent exposure, sexual assault, stalking or obscene communications.

Sexual harassment is not sexual interaction, flirtation, attraction or friendship which is invited, mutual, consensual or reciprocated.³

Sexually disinhibited behaviour: is difficulty controlling sexual behaviour where sexual thoughts, impulses or needs are expressed in a direct or disinhibited way such as in inappropriate situations, at the wrong time or with the wrong person. This behaviour arises for a variety of reasons that can include a mental illness or disorder, cognitive impairment, alcohol intoxication, illicit drug use or the side effects of medication.⁴

Consensual sexual activity in an inappropriate context or setting: for the purposes of the guidelines, consensual sexual activity is any activity of a sexual nature

(sexual touching, sexual intercourse, anal and oral sex) that occurs between people over the age of 16 years after mutual sexual consent has been provided by those involved, who have been assessed as having the capacity to consent. This does not apply to children under 16 years who are in an inpatient or day hospital setting.

The term inappropriate context or setting refers to consensual sexual activity taking place in an environment or associated with a set of circumstances that is not considered to be suitable such as an acute inpatient setting or a non-acute residential or rehabilitation unit. The [CAMHS Consent Guideline](#) provides further information regarding the concept of consent.

Professional boundaries: invisible structures imposed by legal, ethical and professional bodies that determine the limits of a relationship between a CAMHS employee and the child and family. They define the effective and appropriate interactions between professionals and the public they serve. Professional boundaries protect the space between the clinician's power and the child's vulnerability.

Boundary crossings: are brief excursions across boundaries that may be inadvertent, thoughtless or even purposeful if done to meet a special therapeutic need. Boundary crossings can result in a return to established boundaries but should be evaluated by the clinician for potential adverse patient consequences and implications. Repeated boundary crossings should be avoided.

Boundary violation: when the relationship between a health professional deviates from the basic goal of care and treatment. When this occurs, it becomes nontherapeutic and exploitative.

Principles

- All staff are responsible for maintaining professional boundaries between themselves, children and their families. All staff should comply with the [WA Health Code of Conduct](#), [Commissioner's Instruction No. 7: Code of Ethics](#) and maintain awareness of standards and recommendations relating to their own professional body.
- All children and their families and CAMHS staff members have a right to be sexually safe.
- Services within CAMHS support children, their families and staff to adopt practices and behaviours that contribute to their sexual safety, both within the mental health service environment and the broader community. Positive relationships and mutual respect between staff, between staff and children, between staff and parents/carers and between children themselves are encouraged and modelled.
- Services within CAMHS foster a compassionate, sensitive and open culture that encourages reporting of incidents relating to the sexual safety of children, their families and staff.
- CAMHS services are delivered in an environment in which clear information is shared and readily accessible for children, families and advocates regarding individual rights and responsibilities, acceptable and unacceptable behaviours,

and the systems of appropriate avenues for complaint, recourse and redress regarding issues of sexual safety.

- The multiple vulnerabilities of children who access CAMHS are considered throughout the care pathway, whether due to age/developmental stage, potential for cognitive impairment from mental illness, past history, emotional turmoil, or need (Cook, 2001, Goodfellow and Camilleri, 2003).⁵
- A sexually safe environment:
 - Is trauma-informed, caters for sensitivities around gender and sexual orientation and emphasises recovery;
 - Balances personal autonomy and decision making with a duty of care to provide a safe and therapeutic environment for all children;
 - Recognises the common needs of children for privacy and personal space, and that these needs vary between individuals, regardless of gender identity or sexual orientation;
 - Recognises the need for routine identification of sexual risk in all children who receive care from CAMHS;
 - Promotes treatment and early recovery, and actively manages potential disruptions to treatment and wellbeing as a result of breaches of sexual safety;
 - Promotes personal self-care, respect, resilience and self-determination;
 - Responds sensitively to disclosure of past or current sexual assault or abuse; and
 - Listens and responds sensitively and appropriately to all concerns raised about sexual safety.
- Due to the age and vulnerability of the children who attend CAMHS, any sexual activity in CAMHS settings is inappropriate.
- It is illegal for a staff member to engage in sexual activity with a child.
- Staff are strongly encouraged to always seek advice and supervision with any concerns they have regarding a child's sexual safety.

Professional and Therapeutic Stance of CAMHS Staff

- Professional boundaries are integral to the therapeutic care of children and families. They promote good clinical care, protect all parties and maximise trustworthiness (Fallot and Harris, 2009)⁶. Professional boundaries are demonstrated in the following ways:
 - Clearly established limits that allow for safe connections between service providers and their child;
 - A clear understanding of the limits and responsibilities of the clinician's role; and

- Being friendly, not friends.

Assessing Changes in Professional Boundaries

- A number of signs can indicate that professional boundaries between a clinician and child are shifting. A clinician or staff member:
 - Finds themselves giving personal or irrelevant details about their own life;
 - Becomes aware of a strong attachment to a child;
 - Sets aside a lot of time for one particular child;
 - Stays back after hours with a particular child on a regular basis;
 - Becomes aware of strong feelings of dislike for a child;
 - Meets with a child socially on a regular basis;
 - Becomes aware that a child will do whatever they suggest, without question; and
 - Becomes aware of sexual attraction to a child.

Strategies to Help Clinicians Maintain Professional Boundaries When There is Potential for Boundary Violations

- The primary responsibility for boundary protection lies with the clinician.
- All staff have a responsibility to seek advice and raise any concerns with service managers and/or other staff involved.
- Take a proactive approach to any potential boundary violations.
- Discuss strategies with their line manager or service manager such as arranging to share the case with another worker and/or altering workplace arrangements so that the child is moved to a new worker.
- Review your relationship with the child and act to:
 - Respect the parameters of your role in relation to the child;
 - Develop a clear understanding with the child about professional boundaries; and
 - Re-establish professional boundaries with the child by clarifying your role.
- Reflect in personal supervision or consultation:
 - Think carefully about the best interests of the child. Are they being served?
 - Think on the parameters of your role in relation to children. In relation to the potential infringement, is there anything you could have done differently or proactively now to support sexual safety?
 - Take time to reflect on the quality of relationships outside of work. Do you have time to develop and nurture these relationships?

Strategies to Minimise Boundary Infringements in Online Settings

- Staff should not search for consumers or their family members on the internet. Exceptions may be made during times of crisis however it is essential that any such search is conducted with the client's best interests as the primary motivation, rather than the curiosity of the clinician (APS, 2011).⁷ If a staff member feels that using a search engine is a clinical necessity, they should document any occurrences in the client record, providing clear client-centred justifications for conducting the internet search and discuss the need for use of a search engine with their line manager.
- Staff who use social media platforms for personal use are reminded that they need to adhere to the [WA Health Code of Conduct](#), [Commissioner's Instruction No. 7: Code of Ethics](#) and the [CAHS Social Media Policy](#). Most professional ethical and professional guidelines provide recommendations relating to the use of social media. Clinicians must take care to maintain professional boundaries online.
- Staff should not connect with or accept children, their family members or other support people as 'friends' or connections on social media, networking or dating sites as they could be at risk of breaching their therapeutic relationship. If a child requests to be a 'friend' on social media, 'ignore' the request online and discuss this with the child in person, reinforcing the need to keep a professional boundary.⁷

Impacts of Sexual Abuse, Assault, Harassment or Inappropriate Sexual Activity

- The relationship between childhood abuse and the development of mental health problems is well known (Adults Surviving Child Abuse, 2012). Mental health professionals need to be aware of possible past sexual abuse and how this may increase a child's vulnerability to further inappropriate sexual activity, exposure or exploitation by others. The impacts of sexual assault and harassment can be severe and long lasting. The CAMHS Sexual Safety Training package provides background information for staff; the [Sexual Assault Resource Centre \(SARC\)](#) and the [Child Protection Unit \(CPU\)](#) provide valuable resources for clinicians and the general public.

Assessing Sexual Vulnerability, Risk and Safety in CAMHS Settings

Assessment of sexual risk

- Assessment of a child's sexual risk should include present or past history of exposure to abuse or inappropriate sexual activity as part of a developmental history in line with the [CAMHS Clinical Assessment Policy](#) and Statewide Standardised Clinical Documentation (Initial Assessment, page 3). Assessment may garner information from a variety of sources depending on the age of the child or adolescent and should provide sufficient information regarding:

- Basic sexual history including inquiry regarding sexual development, sexual and gender orientation, HIV status and any other sexually transmitted diseases;
 - Vulnerability to sexual assault or harassment;
 - Potential for sexual activity due to illness, emotional state or personality factors;
 - Potential for sexually inappropriate behaviour;
 - Any concerns relating to visitors that may contribute to the person's vulnerability;
 - History of inappropriate sexual behaviour, especially during previous admissions;
 - History of violence; and
 - History of sexual assault or harassment.
- Any disclosure regarding sexual activity in children under 16 years should prompt the clinician to seek supervision. Children in community settings should be assessed for the sexual safety of their current care environment. If significant risks are identified, a notification to the Department for Child Protection and Family Support should be made, consistent with current legislation and in accordance with mandatory reporting procedures. Children in hospital settings should be assessed for the sexual safety of their care environment. This will include assessment of mental status and level of understanding of the rules on the unit.
 - As prescribed by the Children and Community Services Act 2004, mandatory reporters of child sexual abuse include doctors and nurses who are required to make a report when they have formed a reasonable belief that child sexual abuse has occurred. The Guidelines for Protecting Children 2015 identify a duty for all health professionals to manage child abuse.
 - Assessment of a child's sexual vulnerability should be a component of psychiatric and risk assessment along with other risks such as risk to self and others, risk of violence, physical health risk or other vulnerability (see [CAMHS Risk Assessment and Management Policy](#) and State-wide Standardised Clinical Documentation: Risk Assessment and Management Plan (RAMP)). In the RAMP include such items against risk type "F" and any further detail on page two of RAMP (see "Vulnerability in terms of personality" under "Risk Factors" and "Additional Information Regarding Overall Risk"). When sexual risk and vulnerability are identified, strategies must be implemented to manage the risk. Staff should be alert to this possibility from the time of inquiry in community settings or at presentation to the emergency department or inpatient ward.
 - A number of signs and symptoms can indicate a recent or previous sexual assault. The immediate emotional consequences might include terror, anguish, disgust, personal vulnerability, shock, numbness and denial. The long-term impact can be experienced in many ways including:

- Disturbed sleep, frequent nightmares, flashbacks to the assault;
- Embarrassment, shame, eroded self-esteem, poor body image, weight gain used as a protective mechanism;
- Highly sexualised behaviour, hostility and behavioural problems, self-harming and compulsive behaviours; and
- Relationship difficulties, feelings of isolation, guilt or self-blame, suicidal thoughts and suicide attempts, a range of mental health disorders.

Recognising young people vulnerable to sexual or predatory risk

- Children with a cognitive impairment (including cognitive impairment from mental illness) are vulnerable to abuse, particularly those who are homeless, living in boarding houses or institutional settings (Goodfellow & Camilleri, 2003, Adults Surviving Child Abuse, 2012).
- Conditions such as organic brain syndromes may include sexually aggressive behaviours and require appropriate supervision where this occurs.
- Young females who have a history of physical and sexual abuse tend to have a higher degree of lifetime exposure to sexual abuse and violence.
- Gay, bisexual or transgender young people experience increased vulnerability to sexual victimisation.
- Children who are heavily sedated are also more vulnerable to sexual assault.

Recognising behaviour of those who may pose a sexual risk to others

- CAMHS workers should be aware of potential behaviours in children as well as those that care for them, where they may pose a sexual risk to others. Deliberate tactics to perpetrate sexual assault are used to select and abuse vulnerable or desirable children. These tactics may include:
 - Recruiting these children by means of bribes (money, cigarettes, food, other items) or offering perceptions of care, love and intimacy; or
 - Utilising power/positions of power to gain confidence and trust for the purposes of gaining sexual favours; or
 - Maintaining this accessibility by using threats to discourage the individual from disclosing the assault; or
 - Grooming staff by ingratiating themselves as helpful and suggesting to staff that the individual they assaulted is lying.

Assessing patients who may pose a sexual risk to others

- Within CAMHS settings there are children who have a history of sexually inappropriate behaviour or sexually abusive and offending behaviours. Some patients may pose a sexual risk to others because of their mental illness. For example, a patient with sexual disinhibition may act out their impulses. Others may be more predatory in intent and engage in opportunistic behaviour,

targeting vulnerable patients, which may be a repetition of their own experience. Such individuals need to be closely monitored and regularly assessed throughout their admission and at times of transition, such as movement to high or lower dependency areas, or imminent discharge.

- Additional assessment should be included in the SSCD Initial Assessment, Management Plan and medical record and should include history or reports of the following:
 - Sexual disinhibition or aggression from whatever cause (for example mental illness, substance use, acquired brain injury);
 - Sexual activity in previous admissions or having a history of inappropriate sexual activity or sexually offending behaviour;
 - Past sexual abuse perpetrated against the child;
 - Violent behaviour, threats or intimidation;
 - Domestic violence; and/or
 - Illicit drug and alcohol use.

Management of Sexual Risk

- As with any other risk, all children identified as vulnerable to or at risk of sexual activity, assault or harassment should have a detailed treatment or care plan that:
 - Is identified in the management plan and documented in the medical record;
 - Is known to all staff (not just the treating doctor or primary nurse);
 - Specifies the risk behaviours and/or vulnerabilities;
 - Details the approach to be adopted to protect the child;
 - Describes specific interventions; and
 - Is regularly reviewed by the treating team at inpatient review meetings or multidisciplinary team review meetings in community settings and at handover.
- Where possible the plan should be developed in collaboration with the child and their families and carers. The child should be advised, to the extent their mental status allows, what staff are doing to keep them safe and how they might contribute to their own safety, regardless of the risk level identified. The risk to the child's reputation and potential consequences of their actions should also be considered and discussed.
- Where a child's vulnerability is largely a function of their mental illness, an active approach needs to be maintained to the assertive treatment of their acute symptoms and distressed behaviours.

Management of patients who may pose a sexual risk to others

- Risk factors should be recorded on the SSCD Risk Assessment and Management Plan. They should be discussed at inpatient review meetings or multidisciplinary team review meetings in community settings and at handover.

Assessing risk to staff

- All CAMHS staff have the right to feel safe from acts that compromise or breach their sexual safety while in a mental health service environment or in the course of their work in the community. When assessing risk to staff, clinical teams and individual staff should consider:
 - Any history of false allegation or complaints of a sexual nature;
 - PSOLIS alerts which identify concerns regarding sexual safety of staff; and
 - Any history of boundary violations with previous staff.

Reducing Risk: Procedures for Promoting, Protecting and Maintaining Sexually Safe Practice

- Sexual safety can be promoted by actions which can be implemented at an individual patient level, as well as staff, team and service levels.

Staff procedures

- Staff members must take care not to place themselves in vulnerable settings whereby the potential for allegation may arise, for example, alone with a child in a clinic after hours, or transporting a child alone.
- If home visiting, staff should:
 - Avoid being alone in a house with a child without the presence of carers;
 - Maintain safety measures such as the wearing of duress alarms, and the use of safety check-in calls when visiting the homes of children and their families; and
 - Exit a home visit if they feel as though their sexual safety is compromised and report their concerns to their line manager.

Team procedures

- All teams within CAMHS should identify sexual safety practices appropriate for their setting, in collaboration with all members of the service including staff, children and their families, clinicians and advocates.
- When risk factors are identified, these should be discussed specifically at line management and multidisciplinary team meetings. This includes sexual safety issues, identified in the Initial Assessment (page 3 SSCD), and Risk Assessment and Management Plan (specifically risk type F and items within the “Additional Information Regarding Overall Risk” section).

- Where a child is identified as of particular risk, choose a therapy room with an observation window, a room close to reception and/or keep the door ajar during sessions.
- Highlight at-risk children in handover procedures during episode of care, and between multidisciplinary teams and treating medical staff.

Service procedures (including staff training and data systems)

- All staff, including volunteers, must undergo a criminal record screening and have a valid Working with Children Check.
- Risk alert systems must be used to identify children with a history of sexual risk behaviour, for example an alert notice in the medical record and in the PSOLIS record.
- Staff are provided with training and education to enable them to:
 - Effectively promote strategies to support sexual safety and prevent sexual assault and harassment;
 - Respond appropriately and sensitively to sexual safety issues involving CAMHS staff, children and their families, both within the service environment and within the community; and
 - Integrate trauma-informed care principles into all aspects of treatment.

Reducing Risk: Procedures for Promoting, Protecting and Maintaining Sexually Safe Practice for Inpatient Settings

Patient education and engagement

- Encourage the child's presence or involvement in unit activities and programs to minimise isolation and vulnerability.
- Monitor visitor interactions for inappropriate behaviour, particularly of a sexual nature.
- Familiarise young people with their rights, responsibilities and acceptable use regarding recording and sharing of images of themselves and others online.
- Regulate use of smart phones, digital cameras, recording devices, and use of applications which use location-based services.
- To ensure a sense of sexual safety during admissions, the child should be engaged in safety planning collaboratively with staff.
- Provide specific, gender and sexual orientation sensitive information and education regarding boundaries and reinforce general principles of safety and mutual respect on the ward.
- Identify and record each child's preference regarding the gender of clinician/s and proximity to other patients.

Integration of sexual safety into team procedure

- Include sexual safety issues as a routine item within reviews at multidisciplinary team meetings.
- Specifically identify children and adolescents at risk and ensure information is documented in assessment and risk management. This includes plans to manage sexually disinhibited behaviour.
- The child's preferences regarding their preferred pronouns, the gender of clinicians and concerns regarding contact with other patients must be documented in the care plan, discussed at team meetings or multidisciplinary team review meetings and at clinical handover.
- Consult with family and other services already involved (e.g. Community CAMHS / CPFS / School Psychology Services / DAYS) in formulating a safety plan that incorporates specific aspects of a child's clinical profile, his or her personality features and other vulnerability factors.
- Staff activate a PSOLIS alert, ISOBAR information handover and include an alert on the patient's journey board.
- Implement operational plans to increase staff vigilance and observation at times when staff are less available, for example night time handovers and ward rounds.
- Proactively identify potential for re-traumatising children with trauma histories in the event of seclusion and/or restraint being required.
- Ensure that children who are secluded are adequately clothed, and provided with basic needs including bedding and toilet facilities, as directed by the Mental Health Act 2014.
- In the event of an episode of sexual activity staff must follow the procedure outlined in the section on "Responding to and Reporting Sexual Safety Incidents".

Design and ward procedures

- Provide nurse call buttons as requested in both bedrooms and bathrooms.
- Lockable bedroom doors – doors either locked from the outside if requested, or children able to lock their own doors with staff able to override locks and gain access if required.
- Sensory motion detectors installed in ensembles and activated at night.
- Periodic checks of each child in their bedroom throughout the night, hourly as a minimum, or more frequently as identified in Risk Assessment and Management Plans.
- Installing cameras or observation windows in therapy rooms.

Responding to and Reporting Sexual Safety Incidents

- All disclosures of incident/s that compromise sexual safety must be taken seriously and addressed promptly and empathically, regardless of the identity or affiliation of the alleged perpetrator, and with the utmost regard for the complainant's privacy and dignity, past trauma, diversity, and/or the nature of their illness.
- If a child reports an alleged sexual assault, or a clinician suspects that a child has been coerced into engaging in sexual activity or was unwell at the time the activity occurred, staff must inform their immediate line manager and the consultant psychiatrist.
- For patients on Ward 5A, the [Child Protection Unit \(CPU\)](#) at PCH must be contacted to attend the Ward if there is a sexual assault, or reasonable suspicion of a sexual assault (even if the child is non-disclosing).
- Any allegation of sexual contact in an inpatient setting or allegation of sexual assault is a serious incident and must be reported through the mental health service clinical reporting system (Datix CIMS) as soon as practicable (within 48 hours) and investigated, as detailed in the [Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#) and the [WA Health Clinical Incident Management Policy](#). This should include involvement of the Child Protection Unit to ensure independence of investigation.
- Section 254 of the Mental Health Act 2014 describes the duty of staff members to act upon the following reportable incidents:
 - Unlawful sexual contact with a person who is being provided treatment or care by the mental health service by a staff member; or
 - Unlawful sexual contact with a person that occurs at a hospital.

These incidents (or reasonable suspicions of such incidents) must be reported to the person in charge of the mental health service or the Chief Psychiatrist.

- [Responding to an Allegation of Sexual Assault disclosed within a Public Mental Health Service Policy](#) includes best practice responses to support both the alleged victim and alleged perpetrator as well as clinical and administrative procedures (see Section 4.7 for persons aged under 18 years).
- The [CAMHS Sentinel Event Management Toolkit](#): Guidelines for CAMHS service managers and heads of service, provides practical advice and resources for supporting staff, family and carers which can be applied to serious incidents.
- The outcomes of any investigation, including any causal elements identified and recommendations to address them must be fed back at service and directorate levels and inform ongoing quality improvement activities.

Related CAHS internal policies, procedures and guidelines
Mental Health Act 2014
Chief Psychiatrist WA Standards of Clinical Care 2015
Responding to an Allegation of Sexual Assault disclosed within a Public Mental Health Service Policy
Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist
WA Health Notifiable and Reportable Conduct Policy
WA Health Complaint Management Policy (2015)
Children and Community Services Act 2004
Guidelines for Protecting Children 2015
WA Health Clinical Incident Management Policy
CAMHS Sentinel Event Management Toolkit
WA Health Code of Conduct
Commissioner's Instruction No. 7: Code of Ethics
CAHS Social Media Policy
Working with Children (Criminal Record Checking) Act 2004
References and related external legislation, policies, and guidelines
Adults Surviving Child Abuse 2012 <i>Practice Guidelines for Treatment of Complex Trauma and Trauma Informed Care and Service Delivery</i> . Authors Kezelman C.A. & Stavropoulos P.A.
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Medical Board of Australia. Good medical practice: A code of conduct for Doctors in Australia. Melbourne: Medical Board of Australia, 2014.
Medical Board of Australia. Sexual Boundaries: Guidelines for doctors. Melbourne: Medical Board of Australia, 2011.
Nursing and Midwifery Board of Australia. A nurse's guide to professional boundaries. Melbourne: Nursing and Midwifery Board of Australia, 2010.
Identifying and Responding to Student Sexual Offending . Victoria: Department of Education and Training, 2016.
Western Australian Department of Health, Guidelines for protecting children , 2015.

Useful resources (including related forms)
Initial Assessment: Statewide Standardised Clinical Documentation (available on Healthpoint)
Risk Assessment and Management Plan: Statewide Standardised Clinical Documentation (available on Healthpoint)
Sexual Assault Resource Centre (SARC)
Child Protection Unit (CPU)

This document can be made available in alternative formats on request.

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Standards Applicable:	NSQHS Standards:  NSMHS: 2, 6.5, 8.7, 10.4, 10.5 Child Safe Standards: 1, 2, 3, 4, 5, 6, 7, 8, 9, 10		

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Healthy kids, healthy communities

Compassion

Excellence

Collaboration

Accountability

Equity

Respect

Neonatology | Community Health | Mental Health | Perth Children's Hospital